جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : رانيا رفعت هاشم عبدالرحمن

المحافظة : القليوبيه **الحي / المدينة :** شبرا الخيمه اول **العنوان :** 3ش سلامه خليل - ارض نوبار

رقم البطاقة : 28503101401861 **رقم التليفون :** 01203033082 **تاريخ تسجيل الإستمارة :** 24-10-2026

العمر(تاريخ الميلاد) : 10-03-1985 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي المقر بما فيه

التوقيع:(----------) **التوقيع:(----------)**

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] **قسم العلاج :** الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: رانيا رفعت هاشم عبدالرحمن
National ID: 28503101401861

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Bilateral mammogram and breast ultrasound**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين

PATHOLOGY DEPARTMENT
HISTOPATHOLOGICAL REPORT

Patient name: نسمه غزالي حسن عمر
Request type: مبادره
Hospital no:
Department: صحه مرأه
Referred by: د/مروه حسن
Nature of specimen: tru-cut biopsy
Clinical diagnosis: Left breast mass

Age: 41 years **Sex:** female
Sub request type:
Pathology no.: 500B-2026
Date of result: 4/7/2026
Date of biopsy: 15/6/2026
I.D. Number: 28502180104322

Gross pathology :
multiple gray white tissue cores measured 1cm, totally submitted

Immunohistochemical Technique: Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER- PR- Her2- KI67- Ecadherin

Microscopic picture: Section examined revealed core of tumor showing invasive mammry carcinoma with squamoid feature. The tumor forms, nests and cribriform pattern of anaplastic cells with squamoid features infiltrating among desmoplastic stroma. Tumor cells showing marked anaplasia and poor tendency to from tubules. There is moderate number of mitotic figures. No carcinoma insitu component is seen.

Immunohistochemical Result;
ER: Weak nuclear staining in < 1% of tumor cells (Allred Score 2/8)low.
PR: Moderate nuclear staining of 30% of tumor cells (Allred Score 5/8).
Her2: Strong complete membranous staining of >10%tumor cells (score 3+)Positive
Ki67: Positive nuclear staining of 80% of tumor cell.

Diagnosis: Left breast mass, tru-cut biopsy:
Invasive mammry Carcinoma with squamoid features, Grade III

Signature:
Dr.Mona Mohamed.M.D

Pathology Lab. Manager

BASMA REFAAT M.D

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name : Nasma Ghazali Hassan Omar
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 08-08-2026 08:27
Medical No. : 434478

Tc-99m MDP FOLLOW UP SKELETAL SCINTIGRAPHY

History

41 year-old female patient, who gave history of left breast cancer and referred for skeletal survey.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- Uniform radiotracer distribution is noted allover the surveyed skeleton with no evidences of focal active or cold osseous lesion that would account for bone secondaries.
- Degenerative features are depicted at different levels of the spine and around big joints.
- Uneven focal active tracer uptake is noticed opposite the right superior pubic ramus, for CT correlation.

IMPRESSION:-

- No definite osseous metastasis could be depicted on scintigraphic basis.
- Uneven focal active tracer uptake opposite the right superior pubic ramus, for CT correlation.

Much obliged
Dr. Alaa Nagy, MD,
Nuclear medicine

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Patient's Name:	**Nasma Ghazali Hassan Omar**
Date:	**2026-07-26**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**



MULTISLICE CT EXAMINATIUON OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- A lobulated enhancing soft tissue mass lesion is seen in the upperouter,periareolar region of the left breast with irregular outlines and measures about 49x42mm in dimensions. The overlying skin is diffusely thickened and measures about 8.2mm ,internally.
- Multiple left axillary and left retro-pectineal multiple lymph nodes are seen with loss of hilar fat contents in some of them and the largest one measures about 22x20mm in dimensions.
- Normal right breast.
- **Multiple variable sized benign looking right axillary lymph nodes are seen with preserved their hilar fat contents and the largest one measures about 15x10mm in dimensions.**
- Hyperinflated both lungs.
- A tiny enhancing pulmonary nodule is seen in the middle lobe and measures about 5mm in diameter.
- No other sizable abnormally-enhancing pulmonary nodules ,patchy areas of consolidations or calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- Normal bones of the thoracic cage apart from a tiny sclerotic focus in the antero-inferior aspect of DV11.
- Mildly enlarged right lobe and enlarged left lobe of the beaver-tail liver,measuring about 15.6 cm and 17.8cm in diameters respectively, showing a homogenous reduced parenchymal attenuation and a homogenous enhancement pattern.
- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 9.7 cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, functioning both kidneys, both suprarenal gland, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient's Name:	**Nasma Ghazali Hassan Omar**
Date:	**2026-07-26**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

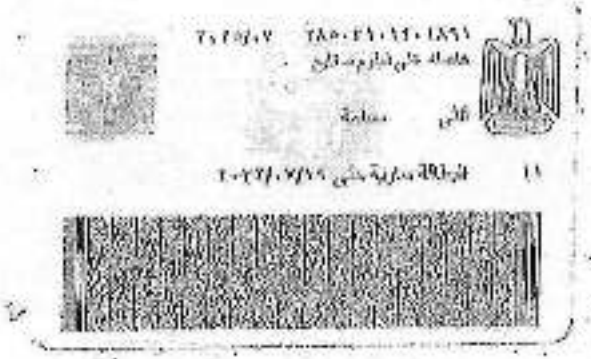
- Bilateral ovarian cyst is seen and each measures about 33x18mm and 35x25mm in dimensions respectively.
- The rest of the pelvic structures are unremarkable.

IMPRESSIONS:

- **A lobulated enhancing neoplastic soft tissue mass lesion is seen in the upper outer,periareolar region of the left breast ,the overlying skin is diffusely thickened and multiple left axillary and left retro-pectineal lymphadenopathy,likely to be metastatic.**
- **A tiny sclerotic focus in the antero-inferior aspect of DV11 and a tiny enhancing pulmonary nodule in the middle lobe,likely to be metastases in evolusion.**
- **Bilateral small ovarian cysts.**

Prof/Hanan El-Ahwal.

CT Scan
[[DOCTYPE:CT_SCAN]]





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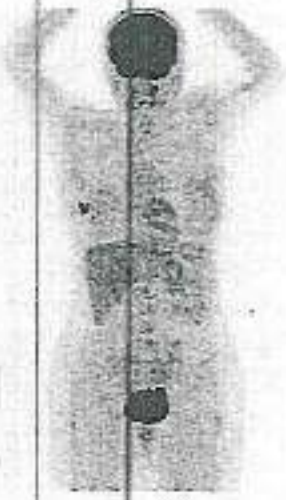
National ID – Front & Back
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NILE
SCAN & LABS

مركز النيل للأشعة والتحاليل

Name : Rania Rifaat Hashem Abd Al. Rahmaan	Date : 02-08-2026 08:31
Branch : Nasrcity	Medical No. : 429283
Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : التأمين الصحي

Whole body ^{18}F-FDG PET/CT EXAMINATION



Clinical history:

- 41 years old patient with right breast IDC.
- PET/CT requested for Initial assessment.

Technique:

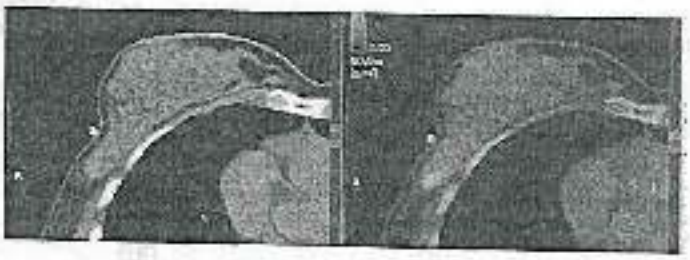
- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using GE Discovery IQ mCT simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV; a Semi-quantitative analysis would be done for selected ROI.s and the normal threshold is < 2.5.

Report:

Reference hepatic activity:2.6

Loco-regional:

Right breast: Dense breast parenchyma showing Abnormal FDG uptake is seen over ill-defined UOQ soft tissue neoplastic infiltration measuring roughly 2.7x1.3cm with **SUVmax=4.4**



NILE DIAGNOSTIC CENTER

PET Scan
[[DOCTYPE:PET_SCAN]]

Name : Rania Rifaat Hashem Abd AL Rahmaan	Date : 02-08-2026 08:31
Branch : Nasrcity	Medical No. : 429283
Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : التامين الصحى

Right axilla: *Low grade FDG avid few axillary LNs reaching 1.1x0.7cm with* ***SUVmax~1.5.***



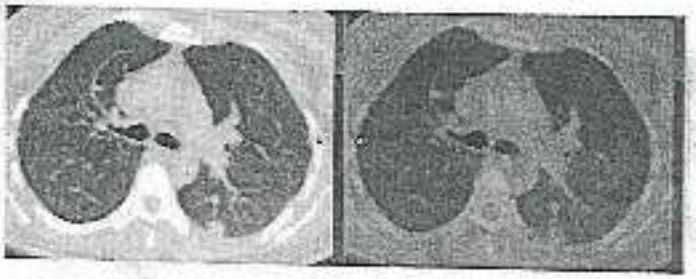
Left breast: *Dense breast parenchyma, otherwise, Unremarkable.*

Left axilla: *No hypermetabolic left axillary LNs detected.*



Distant

Non FDG avid left lower lung pleural based nodule measuring 1.4x1.2cm, ***of suspicious nature for close follow up***



Head and neck:

PET Scan
[[DOCTYPE:PET_SCAN]]

NILE
SCAN & LABS

مركز النيل للأشعة والتحاليل

Name : Rania Rifaat Hashem Abd AL Rahmaan	Date : 02-08-2026 08:31
Branch : Nasrcity	Medical No. : 429283
Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : التامين الصحي

NILE DIAGNOSTIC CENTER

- *Normal diffuse brain activity with no hypermetabolic brain lesions.*
- *Physiologic FDG uptake is seen in the nasopharynx, oropharynx, salivary glands, and larynx, showing normal appearance.*
- *Physiologic uptake is seen in submandibular and parotid salivary glands, showing normal appearance.*
- *Normal size, enhancement, & symmetrical appearance of thyroid gland with No FDG avid lesions detected.*
- *No hypermetabolic cervical lymphadenopathy detected.*
- *No FDG avid neck soft tissue mass lesions.*

Chest:

- *No hypermetabolic mediastinal lymphadenopathy.*
- *No metabolically active pericardial or pleural lesions detected.*
- *Physiologic FDG uptake is noted in the heart.*

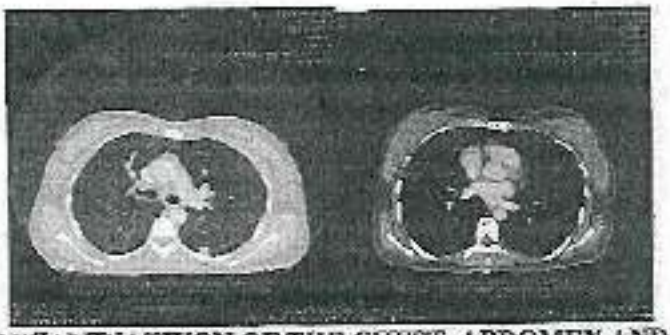
Abdomen and pelvis:

- *Physiologic diffuse FDG uptake is seen over average size liver with homogeneous parenchymatous texture. No FDG avid focal lesions detected. Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries. No intrahepatic or extra hepatic biliary radicles dilatation.*
- ***Tiny gall bladder stone,***
- *Physiologic diffuse FDG uptake is seen over average size spleen with homogeneous parenchymatous texture and no FDG avid focal lesions detected.*

PET Scan
[[DOCTYPE:PET_SCAN]]

جوستاف روسي

Patient's Name:	Rania Rifaat Hashem
Date:	2026-07-13
Referred by:	Prof.Dr. no doctor
Hospital:	Goustave Roussey International Egypt



MULTISLICE CT EXAMINATIUON OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- An enhancing dense lesion with irregular outlines is seen in the lower outer quadrant of the right breast and measures about 27x18mm in dimensions.
- Multiple variable sized right axillary and retro-pectineal lymph nodes are seen with loss of their hilar fat contents and the largest one measures about 13mm in diameter.
- Normal left breast with benign-looking left axillary lymph nodes seen with preserved their hilar fat contents and the largest one measures about 16mm.
- Hyperinflated both lungs.
- A subpleural enhancing nodule is seen in the apical segment of left lower pulmonary lobe and measures about 17x14mm in dimensions.
- Few thin linear atelectatic changes scattering in both lungs.
- No other sizable abnormally-enhancing pulmonary nodules ,patchy areas of consolidations or calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- Normal bones of the thoracic cage.
- Average sized liver,measuring about 14.2 cm, showing a homogenous parenchymal attenuation
- and enhancement pattern.
- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- A sand-like gravel is seen in the gall bladder fossa,measuring about 4.8mm with its CT density of 254 HUs.

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient's Name:	Rania Rifaat Hashem
Date:	2026-07-13
Referred by:	Prof.Dr. no doctor
Hospital:	Gonstave Roussey International Egypt

- Average sized spleen,measuring about 12.3 cm. No focal lesions seen inside.
- Thickened left adrenal gland.
- Normal CT appearance of the pancreas, functioning both kidneys, right suprarenal gland, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- A left ovarian cyst is seen and measures about 62x31mm in dimensions.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.

IMPRESSIONS:

- An enhancing dense lesion with irregular outlines in the lower outer quadrant of the right breast with multiple variable sized right axillary and retro-pectineal lymph nodes,likely to be metastases.
- Hyperinflated both lungs with a subpleural enhancing nodule in the apical segment of left lower pulmonary lobe,likely to be metastatic in natur.
- Few thin linear atelectatic changes scattering in both lungs.
- Thickened left adrenal gland,likely to be a hyperplasia rather than metastases.
- A sand-like gravel in the gall bladder fossa.
- A left ovarian cyst.

Prof/Hanan El-Ahwal.

CT Scan
[[DOCTYPE:CT_SCAN]]